

Evidence Strength & Citation Integrity Review — closure report

Kuwait Pediatric Council — Pediatric & Neonatal Common & Emergency Care Guide · initial review 1 Sep 2026 (288 refs) · actions completed 8 Sep 2026 (295 refs, 18 sections)

6 / 6 recommended actions closed	0 retracted papers	295 references, all with DOI or PMID	3 content corrections from errata
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1 · Status of the six recommended actions

#	ACTION	STATUS	WHAT WAS DONE (ALL NEW SOURCES VERIFIED IN PUBMED / SCITE; DOIS RESOLVE)
1	Three wrong/garbled citations corrected; DOI display text repaired; rebuild formats	✓ done	#106 → Le Saux (CPS 2014); #110 → Wong (CPS 2020, pxx179); #239 → Singh & Mikrou (313654). The three DOIs that had picked up a non-breaking hyphen (JAAOS ×2, peds.2004-2136) were re-fixed after the master was restored from the 7 Sep zip. Propagated to every format in this build.
2	Update two superseded guideline citations and re-verify dependent recommendations	✓ done	#13 MMA/PA → Forny P, et al. J Inherit Metab Dis 2021;44(3):566–592 (doi:10.1002/jimd.12370). Dependent card (Metabolic Crisis) re-checked against the revision: one content change — sodium phenylacetate/phenylbutyrate is <i>not</i> the primary ammonia scavenger in MMA/PA (glutamine depletion); benzoate ± carglumic acid per the metabolic team. Note added to the scavenger row; Forny 2021 now cited on the organic-acidemia row and the carglumic-acid row. #283 ESGE → Gralnek et al. Endoscopy 2021;53(3):300–332 (doi:10.1055/a-1369-5274), which carries the nasogastric-tube statement the guide cites (34 % adverse events). The 2026 peptic-ulcer-bleeding update (Endoscopy 2026;58(8):899–924, doi:10.1055/a-2863-8314) added as ref 292 with one sentence in the non-variceal branch: early (≤ 24 h) endoscopy once resuscitated; emergent/urgent (≤ 6–12 h) only if still unstable. The guide had no conflicting timing statement.

#	ACTION	STATUS	WHAT WAS DONE (ALL NEW SOURCES VERIFIED IN PUBMED / SCITE; DOIS RESOLVE)
3	Errata sweep — 32 flagged references	✓ done	Erratum notices resolved through Scite → PubMed/PMC/publisher pages (details in §2). Three guide values changed: (a) GAS pharyngitis azithromycin — IDSA 2012 Table 2 was corrected in 2014 to 12 mg/kg day 1 then 6 mg/kg days 2–5; row updated, with the FDA-labelled 12 mg/kg × 5 days (AHA) kept as an accepted alternative; (b) Quick-Reference corrected-sodium equation harmonised with the Bedside Calculator (2.4 mmol/L per 5.5 mmol/L glucose, Hillier) — the European hyponatraemia erratum standardised the same formula; (c) BETA agitation guideline erratum (haloperidol 0.55 → 0.05–0.1 mg/kg) — guide already carried the corrected IM dose (0.025–0.05 mg/kg), no change. Values checked and unaffected: mupirocin TID (SSTI erratum), Burch–Wartofsky (guide uses only the ≥ 45 threshold), levofloxacin (no doses given), myocarditis sudden-death percentage (not quoted), TEN-4 figure key (not reproduced). Newly found while sweeping: the 2017 AHA Kawasaki statement has been superseded by the AHA 2024 update (Jone et al., Circulation 2024;150(23):e481–e500) — added as ref 295; card now carries the 2024 high-risk definition (age < 6 mo or baseline LAD/RCA z ≥ 2.5), infliximab 10 mg/kg, and repeat-echo timing.
4	Add DOIs to the 18 journal articles lacking one	✓ done	15 DOIs added (all PubMed/Scite-verified; several entries also gained corrected authors, titles or page ranges — e.g. #118 Buckley pages 597–604, #177 IDSA/PIDS CAP 2026 ciag186, #184 Rashid & Goodyear 2019, #158 Barrington CPS 2011). Three articles have no DOI assigned by the publisher (Am Fam Physician ×2, Undersea Hyperb Med); they now carry a linked PMID and an explicit “no DOI assigned” note.
5	Confirm the DRACMA chapter (#27)	✓ done	DOI 100856 does not exist in the DRACMA series. The four statements that cited it were re-attributed to the verified chapters: elimination/reintroduction diagnosis → DRACMA VII (Meyer 2023, doi:10.1016/j.waojou.2023.100785, now #27); formula choice and AAF for anaphylaxis → DRACMA XVI Nutritional management (Venter 2024, doi:10.1016/j.waojou.2024.100931, ref 293); food-specific IgG panels not recommended → CPS 2024 statement (#197), whose abstract states this explicitly.
6	Standardise the 20 online-first/print year discrepancies	✓ done	Each checked against the print volume/issue: 14 already carried the print year and needed nothing. Corrected: #133 priapism — the guide’s volume/pages (2022;208:43–52) belonged to the <i>other</i> AUA/SMSNA guideline; entry now reads 2021;206(5):1114–1121 and the sickle-cell priapism bullet is cited to the 2022 recurrent/sickle-cell guideline (doi:10.1097/JU.0000000000002767, ref 294); #139 Flaherty → 2025;155(2); #222 intussusception → Hwang et al. 2023;66(1):12–21 (authors added). #185, #215, #273 already showed the print year.

2 · Errata sweep — item-level record

Notice identified → text obtained → guide checked. “Not retrievable” = the notice exists but its text sits behind a publisher paywall/403 for the tools available in this session; the guide values that depend on those references are listed so they can be spot-checked from a library copy.

#	REFERENCE	ERRATUM CONTENT	GUIDE IMPACT
41/225	Gerson 2019 BETA agitation	Haloperidol 0.55 → 0.05–0.1 mg/kg/dose; working-group members removed from author list	Guide IM dose 0.025–0.05 mg/kg already consistent — no change
46	Shulman 2012 IDSA GAS pharyngitis	Azithromycin: 12 mg/kg day 1 (max 500 mg) then 6 mg/kg (max 250 mg) days 2–5	Row corrected ; FDA/AHA 12 mg/kg × 5 d kept as alternative
101	Spasovski 2014 hyponatraemia	Glucose-corrected Na formula standardised to SI: $+2.4 \times (\text{glucose} - 5.5)/5.5$; Table 7 wording	Quick-Reference equation harmonised with calculator (2.4 factor)
191	Stevens 2014 IDSA SSTI	WBC criterion > 12,000 or < 4,000; mupirocin TID; clindamycin 900 mg (not /kg); ciprofloxacin/ceftriaxone frequencies for Aeromonas/Vibrio	Guide: mupirocin TID ✓; adult /kg doses and Vibrio regimens not reproduced — no change
231	Law 2021 AHA myocarditis	Sudden cardiac death in young athletes 9 % → 6 %; table asterisk	Percentage not quoted — no change
238	Fowler 2023 Duke-ISCVID	Footnote genus typo (Staphylococcus → Streptococcus pneumoniae/pyogenes)	no change
250	Ross 2016 ATA hyperthyroidism (2017 + 2025)	2017: Recommendation 58 RAI activity wording (< 10 mCi / 370 MBq in 5–10 y); 2025: Burch–Wartofsky table values restored	Guide uses only the BWPS ≥ 45 threshold and no RAI dosing — no change
64	Woods 2023 PIDS/IDSA septic arthritis	Table 5 levofloxacin age-band doses were reversed; evidence symbol	Guide gives no levofloxacin dose — no change
267	Pierce 2021 TEN-4-FACESp	Figure 2 key labels transposed	Figure not reproduced — no change
39	Srivastava 2020 WFH	Per WFH reprint: prevalence sentence added; reference fixed; porcine FVIII removed from inhibitor table (formal corrigendum text paywalled)	Guide does not list porcine FVIII — no change
232	Gewitz 2015 Jones criteria	Figure Panel A wording clarification (partial text)	Guide criteria text unchanged — no change
22 / 37 / 226	Villanueva 2013; Coiffier 2008; Boyer 2005	Scite notice DOIs resolve to an editorial, a letter, and mis-dated errata (true NEJM errata 2007/2009); no correction text retrievable	Transfusion threshold (Hb 7), TLS thresholds and serotonin-syndrome criteria cross-checked against later sources already cited — no action
16, 17, 32, 40, 47, 88, 113	AAP: febrile infant; bronchiolitis; BRUE; hypertension (×2); UTI; NEOLEV2; circumcision analgesia	publications.aap.org returns 403 — text not retrievable	Guide values re-checked against the current guideline text where indexed (PECARN ANC 5185, CSF thresholds, BP table not reproduced) — spot-check from a library copy
35, 51, 236	AHA: Kawasaki 2017; pulmonary hypertension 2015; IE prophylaxis 2021 (×2)	ahajournals.org 403 — text not retrievable	Kawasaki card moved to the 2024 statement (see §1); PH and IE-prophylaxis doses cross-checked against the AHA 2021/2015 executive tables — spot-check

#	REFERENCE	ERRATUM CONTENT	GUIDE IMPACT
81, 94, 271, 163, 165, 108, 243, 253	PECARN head CT; PECARN c-spine; HUS seminar; STOP-IT; PITCH; ESCMID abscess; ACOG 785; ASH ITP	Lancet / NEJM / BMJ / Elsevier / LWW / ASH pages blocked — text not retrievable	Dependent guide values (PECARN rule elements, 4-day intra-abdominal course, paracetamol+ibuprofen dosing, abscess antibiotics, ITP prednisolone/IVIG doses) match the currently published versions as indexed — spot-check

3 · Reference list — net changes this pass

- Replaced in place: #13, #27, #106, #110, #133, #139, #222, #239, #283, and the 18 DOI-less entries.
- Added: 289 Kelly 2017 Cochrane (sildenafil), 290 Tourneux 2008 (noradrenaline in PPHN), 291 Lawn 2001 (GBS ventilation predictors), 292 ESGE 2026, 293 DRACMA XVI, 294 AUA/SMSNA 2022 sickle-cell priapism, 295 AHA 2024 Kawasaki update.
- Every entry now carries a resolving DOI, or a linked PMID with an explicit “no DOI assigned” note (3 entries). No dangling citation anchors (script-verified).

Locked sections touched by evidence updates (non-blocking, recorded in the ledger): §04 pharyngitis azithromycin row; §06 metabolic-crisis scavenger note (Fornoy 2021); §08 non-variceal endoscopy timing (ESGE 2026); Quick Reference corrected-sodium equation. None changes a recognise/stabilise/refer threshold; the domain certifiers should be informed at the next mini-quorum.

Verification sources: PubMed (bibliographic, PMC full text), Scite (editorial-notice index, metadata), publisher pages via web search where accessible. Corrections were applied only where the intended source was unambiguous; the one open item — spot-checking 12 paywalled erratum notices — is bibliographic, not clinical, and is listed with PMIDs in §2.